# Expansion of Chowgacha Jhenaidah (C-J) Community Engaged Health Management Model

Draft National Strategic Plan 2019-22







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[In case of different meaning between English and Bangla version, Bangla version is the original version]

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# Expansion of Chowgacha Jhenaidah (C-J) Community Engaged Health Management Model

## Introduction

Community engaged health management is such a system through which individual, family and community takes responsibility of health and welfare for own and community. In this way they are able to contribute towards community health development. In this type of management people (a) participate in planning, implementation and evaluation; (b) support in structural development of institution, manpower, procurement of medicine and supplies; (c) build awareness and integrating with healthcare, it helps to utilize health services.

Bangladesh is committed to implement Universal Health Coverage (UHC) by 2032 like other countries of United Nations. By UHC every citizen of Bangladesh will receive quality healthcare. But it may be a challenge to provide quality healthcare while securing economic security to all the people of the country. It has been found in studies that, 80% people do not get health service from government hospital, only 23% of the maternal deliveries are conducted in government hospital. The Upazilla Health Complex Hospital are underutilized mostly. To face these challenges, necessary resources need to be invested in district and upazilla level hospitals as well as hospitals at any level throughout the country, to attain quality improvement in hospital management and reach the desired goal of performance. Implementation of universal health coverage is not possible unless: standard improvement of the hospital management; ensuring transparency and responsibility of health providers; organizing health manpower in a team spirit; regular supply of manpower, logistics, equipment, medicine and other necessary health components; community and administration's engagement etc.

The bright example of community engagement in government services is the successful EPI campaign and Community Clinic operation. The successful milestone of such example is Chowgacha Jhenaidah (C - J) community engaged health management model. The C-J Model is such a healthcare which is based upon morality, under the leadership of skilled person and team committed to people's welfare, ensures optimal utilization of government resources by local management. The model is approved by the Bangladesh Government and World Health Organization. The main components of C-J model are target oriented activities, optimum utilization of government resources and support from the community, involvement of the community, ownership development. Many of the hospitals like Chowgacha and Jhenaidah implemented community engaged system, the managers having the quality of leadership, dedication for health service, to attend the hospital full time, well behaviour, transparent economic management, provide health service in a team, goal oriented health activities,

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collecting the resources locally, if needed; to address manpower and equipment unavailability, and to maintain relationship with the local people and administration etc. These health facilities solve their problems easily by the support of the community. The other components of the model are, provision of manpower and supply of necessary health equipment with the help of local people and the private donors. Moreover, ensuring safety of the service providers (Doctors, Nurses and other health workers) and creating people-friendly workplace. In this health management model, local people's representative, local administration local physician, lawyers, journalists, police, teacher, NGO and industrialist and wealthy people of the community, students and representatives of different class-profession are involved. Where the model has been replicated partially or completely, there the local resources are mobilized and utilised those optimally, various types of corruption have been prevented and overall patient friendly environment have been created; whereas healthcare quality have been improved exemplarily.

By taking above initiatives the performance of Chowgacha Health Complex increased significantly. From a statistical data of Chowgacha Health Complex, it has been shown that bed occupancy rate of Chowgacha UHC was 78.9% in 1997 which improved to 208% within 2011. First visit of antenatal care was 16.3% (817) in 1996 increased to 170% (8935) in 2011, hospital delivery rate was 2.1% (99) in 1996, to increase 98.4% (4413) in 2011, Caesarean section rate was 1.02% (01) in 1996 which increased to 22.5% (1007) within 2011. Community engaged health management model enabled improvement of Chowgacha UHC's important health indices significantly. In Chowgacha UHC maternal mortality rate was 599/100,000 live births in 1995 which reduced to 68/100,000 live births in 2011. Neonatal mortality rate and under 5 mortality rate were 42/1000 live birth and 78/1000 live birth respectively in 1995 to decrease 18.2 and 23.6 in 2011 respectively. Though the performance of the health complex is declining in 2017 due to various reason but the latest performance is higher than any other Upazilla Health Complex of the country. In Jhenaidah Sadar Hospital after implementing the model, the number of healthcare seekers at emergency and outdoor of the hospital were 81,795 in 1998 which increased to 340,932 in 2017. In indoor admitted patient number were 12,973 in 1998 which were 35569 in 2017. The bed occupancy rate was 126% in 1998 which were 242% in 2017. Total number of hospital delivery were 1106 increasing to 2997 in 2017. The hospital delivery in Jhenaidah Sadar hospital was the highest among all the government hospitals of Khulna Division in 2017. In case of hospital delivery number, Chowgacha Health Complex stood first in 2011 and Jhenaidah Sadar Hospital stood first in 2017 among all the government hospitals in Khulna Division.

In addition to above mentioned two hospitals, many of the hospitals improved their quantity and quality of the services following the Chowgacha- Jhenaidah (CJ) model. Those countries who achieved universal have coverage, community engagement and involvement of local

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people were important factors of their success. The Government of Bangladesh is trying to achieve universal health coverage by replicating CJ model throughout the country.

## Evolution of CJ model

Chowgacha Jhenaidah community engaged model evolved in 1985 in Upazilla Health Complex of Chowgacha, Jashore district. Freedom fighter Dr. Humayun Mollah was the Upazilla Health and Family Planning Officer (UHFPO) of the Upazilla Health Complex (UHC). A group of young doctors worked under him. The team improved the outdoor and field services of the Upazilla. Dr Md. Emdadul Haque was one of the doctors working in the team.

Though limited resources were available, they faced their challenges and improved the health service system by their innovative activities. In 1986 with the help of Upazilla Parishad, they shifted and expanded the Health Complex from a tally shed to the nearby public library building and opened indoor ward of six beds. A building was also constructed by Upazilla Parishad for EPI program. Eye camp, diabetic centre, ANC corner and other activities got a kick-off with the help of local administration, clubs and community. Within a short period, the Health Complex services became very popular. The Health Complex was awarded nationally for its best performance for EPI in 1989. One of the important components of the health complex was 'zero tolerance to corruption' in health service. The role of this was of paramount importance to earn confidence of local community.

Community engaged health system expanded to Lohagara Upazilla Health Complex, Narial in 1989-1995, under the leadership Dr. Md. Emdadul Haque, who improved the health and family planning services of the upazilla. He drafted a Lohagara development plan involving local community leaders, teachers, social workers, press reporters. Special among them were high officials and dignitaries who were born in Lohagara.

Six bedded Chowgacha Upazilla Health Complex was shifted to 31 bedded new Health Complex building in 1994. Freedom Fighter Dr. Abdul Mannan was serving as Upazilla Health and Family Planning Officer (UHFPO) at the same time. He also started six bedded maternity ward in 1994 for basic Emergency Obstetrics Care (EOC). Dr. Emdadul Haque returned to Chowgacha Upazilla Health Complex in 1995 with the experience of community engagement at Lohagora. He along with the health authority, started health education corner, ORT Corner, EPI Corner at the UHC. The data collection of birth and death were launched throughout the upazilla. This was not a usual practice in the health complex. In 1995-1998, Mother and Child Health development program was implemented at Chowgacha Health Complex with the help of Japan International Cooperation Agency (JICA) under Frontline MCH Program. They provided logistics and technical support.

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In August 1996, Dr. ASM Abdur Razzaq and Dr. Selina Khatun joined the Chowgacha Health Complex. With their cooperation, taking local initiative, upazilla health authority started antenatal care, under 5 corner for under 5 children, safe blood transfusion services for pregnant women to fulfil the needs of improving maternal and child health. A base line survey was conducted and a report was prepared in January 1998. At the same time local administration, industrialist and community played a role to improve sanitation and prevention of child marriage in the Upazilla. All the 8 (eight) components of primary health care was effectively implemented in Chowgacha Health Complex, which was topmost among all. People-friendly innovation and community engagement were the key point of success. In 1998 eight (8) nurses of Chowgacha Health Complex were trained on EOC from Ad-din Hospital, Jashore under Frontline MCH program. It may be mentioned, at that time the nurses were not so experienced as today, on management of pregnancy care. With the help of UNICEF, comprehensive EOC service started in Chowgacha UHC in 2000. A new building for pregnancy care was started functioning with the manpower recruited with community support. In this way the model of community engaged health management developed step by step.

A plan of action was developed locally in 2001 for the development of maternal and child health by 2010, known as vision 2010. It was an exceptional example for upazilla health management. In 2008 the Health Complex was upgraded from 31 to 50 bedded hospital and the Ministry of Health and Family Welfare declared the Health Complex as the Model UHC of Bangladesh. In 2010 Dr. Margaret Chan, Director General of WHO, visited Chowgacha UHC and recommended to expand the model throughout the country.

On 1 January 2011, Chowgacha UHC started Non-Communicable Disease (NCD) prevention programme by creative initiative of local health authority, which was first among upazillas. In December 2011 at Chowgacha UHC, hospital delivery rate was more than 90%, due to special action to improve maternal health. To recognize the setting up example for improving health services, Ministry of Health and Family Welfare started to upgrade the hospital from 50 bed to a 100 (hundred) bed facility in May 2017.

Dr. Md. Emdadul Haque joined in Jhenaidah Sadar Hospital as Senior Consultant (Obs & Gynae) in May 2012. Though Jhenaidah Sadar Hospital was upgraded from 50 to 100 bed hospital in 1994, but post for manpower was not created during the long 18 years. But patients in hospital increased to three-fold. There was no post of aya, ward boy, guard, gardener, trolley-man, and electrician in the hospital. The required number of government cleaner was 36, but available was only 6. So the hospital was suffering from inadequate manpower, lack of cleanliness, insecurity, mismanagement and other problems. At that time, training program of 5S Kaizen system was ongoing by Savar Public Administration Training Centre (PATC) to improve healthcare services. At the initiative Dr Emdad, Jhenaidah hospital engaged community, and

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overall improvement in environment and quality of care achieved by following the Chowgacha model. In 2014, the hospital started overall improvement under “improving safe motherhood” program of JICA. As a result, The hospital was recognized as the best quality hospital among district hospitals of the country in 2016. In 2015 Health Economics Unit (HEU) of MoHFW took the initiative of piloting the Jhenaidah District Hospital as a Model District Hospital, and formed organizational structure of the model. In 2016, Quality Improvement Secretariat (QIS) of HEU replicated this model in 10 District Hospitals, and now 11 district hospital and 5 UHCs are replicating it throughout the country.

## Characteristics of C-J Community Involved Health Management Model

1. Successful leadership in health management
2. Personal and group skills in health care
3. Community engaged health management
4. Optimum use of government resources
5. Effective relation with local administration and with other government organizations
6. Improvement of security by community engagement
7. Improved maternal health care activities
8. Improved child health care activities
9. Ensuring corner-based health care
10. Birth and death recording, progress assessment and accountability by collecting information on all health indicators
11. Improving healthcare by unit management
12. Health awareness by health education
13. Cleanliness (Waste management) and beautification of the facility and premises
14. Provision of safe blood transfusion for emergency needs
15. Zero tolerance against corruption
16. Implementation of the 5S KAIZEN-TQM for the improvement of health care
17. Improved financial management by establishing cash counter

### 1. Successful leadership in health management

The role of leadership in the evolution of the C-J model was unrivalled. Successful implementation of public health management has been possible by successful application of honesty, sincerity, sympathy, empathy, etc. Leadership played unique role in this model. However, single as well as collective leadership, co-operation and coordination of like-minded colleagues played an important role in implementing this model. The UHFPO of the Chowgacha UHC and his colleagues played a unique role in this regard. However, in Jhenaidah single leader played the main role.

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The success of this model depends on the role of a leader and a skilled team. Initiative of the individual is an important pre-condition for the implementation of the model.

## 2. Individual and group skill in health care

Successful staging of a team effort in health care, individual and teamwork skills as well as quality of performance of all types of employees, including doctors, nurses, are very important. A successful leader encourages the team to form, manage and achieve desired success. In Jhenaidah and Chowgacha, the active role and positive attitude of health care providers and organizations are contributing to the development of the model.

Their initiative has been encouraged by the concerned healthcare seekers and healthcare providers. Continued cooperation and encouragement of the MoHFW and Director General of Health Services (DGHS).

## 3. Community participation in health management

Health is wealth. Ensuring health is a right of every individual. To improve and sustain the wealth, community participation should be considered essential. Active participation in Identification of health problems and priority selection and local planning for implementation; and mobilizing necessary resources to operate essential services is possible through community engagement. As for example, 31 manpower at Chowgacha Health Complex and 56 support manpower at Jhenaidah Sadar Hospital was received through community support. Even after optimum utilization of the government resources, scarcity of resources remains. To replenish it, community engagement may play an important role. This role has scored success at Chowgacha, Jhenaidah and other health facilities at District and Upazilla, which has been considered as examples.

Even community engagement play role in increasing utilization of health services. By community engaged health management people play in improving their own healthcare through (1) participating in planning, implementation, management and evaluation (2) supporting development of institution structure, recruiting manpower and procurement of necessary equipment; and (3) building awareness; and also play role in utilization of health service through involvement in healthcare. By community engaged C-J Model, bed utilization has increased, increased confidence in government health system helped in expansion of government service coverage and access, as well as decrease in mortality rate which positively affected the health indicators. The peoples' representative, political leaders and civil society may play important role in community engagement. At the same time, cooperation and coordination with other government departments is another important component of success of this model.

Let us cite an example. The Officers Club headed by Upazilla Nirbahi Officer (UNO) exist at Chowgacha Upazilla. All officers including doctors and Professors of local colleges are members

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of this Officers Club. So, opportunity is opened for working together to play their role in government functions and social development. At Chowgacha, the Officers Club played role in organizing Eye camp, scholarship foundation for students, preventing copying at public examination, preventing child marriage, collecting vital statistics and safe motherhood services. As a result, close connection developed between health complex and other government officials and employees.

Similar to Officers Club, cordial relation with Press Club also developed. They assisted to create public opinion locally in favour of hospital. At both Chowgacha and Jhenaidah, journalists fully maintained relation with health facilities.

## 4. Optimum utilization of government resources

The government resources are: human resource, money, facilities, working hours and others. One of the characteristics of the model is to ensure the optimum use of these resources with honesty and responsibility. For this reason, people came forward to replenish the deficit of resources.

## 5. Participation of local administration and other government organizations

Local public administration paly a very important role in any program implementation at local level. Under the leadership of Chairperson of Upazilla Parishad, officials of Upazilla Parishad take responsibility to implement health and other relevant peoples' welfare programs. The Pourasavas wherever exist among Upazillas, are similarly mandated for improving health of their citizens. It is essential to involve the Pourasavas for support in resource mobilization, active participation, supervision, monitoring in various health activities; combating health risk or epidemic due to natural or man-made calamities; and protecting healthcare providers and health facilities and other activities. The inter-departmental coordination between public administration, police, education, social welfare and other government departments is very important for improving health system. In improving health system of C-J model, other government agencies including public administration are playing a very important role.

## 6. Improvement of security by public relations

Recently security in the hospital has become a thorny issue. Different types of dissatisfaction of the patient or their relatives have surfaced up on and often. At some point isolated, scattered, sporadic attacks by the patients' relatives are occurring at the hospital. Even in district hospital due to lack of adequate security service, healthcare providers are often confronted with safety issues. In C-J model, the round-the-clock security have been ensured in this ideal management. An important part is to utilize the volunteers to ensure safety of both healthcare seekers and providers. Due to the activities of the Community Support Group (CSG), the public relations have created a new confidence-building environment for both doctors and patients.

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## 7. Improved maternal health care services

The motto of Chowgacha Health Complex was to ensure all pregnant mothers would take antenatal care directly from the health complex at least 5 times, and 100% of mothers are to receive care from the hospital. As a result of repeated antenatal visits in the hospital, the relationship between the service provider and the mothers were developed. So, confidence upon the hospital services grew. As a result, pregnant mothers became enthusiastic and courageous for institutional delivery. By providing EOC services with utmost efficiency and sincerity, the rate of normal delivery has increased and the caesarean section decreased. Subsequently, the rates of maternal mortality and infant mortality decreased substantially. One of the prerequisites for improved maternity care is the safe blood transfusion in emergency. Health authority encourages the local people to donate blood and arrange immediate blood collection and blood transfusion during urgent need. Emergency care services are available for emergency mothers (24/7). To ease the tasks, the shortage of manpower has been solved by volunteer (nursing assistant) mobilization supported by the community. Extra bed arrangements have been made for maternity. The environment was women friendly, privacy and confidentiality were maintained. Since 1996, for the poor pregnant mothers attending Chowgacha UHC, the "Matir Bank'' (indigenous bank made by earthen pot) programme encouraged women to save money every day, for use at future emergency.

## 8. Improved child health care services

At the local initiative, Under 5 corner initiative started since 1996, for the children aged under five years, at Chowgacha UHC. It was officially introduced under the name of IMCI Corner in 2007. In Under 5 corner, growth charts are used for measuring the nutritional status of the children. From here, all the children are provided a primary treatment, including knowledge about EPI, diarrhoea, breast feeding, malnutrition. All the children from here are referred to a designated physician. It is to be noted that the separate child ward was opened in the indoor of the UHC since 1996. These activities of Chowgacha is running continuously till date. Due to these services child mortality rate and extent of malnutrition came down. Neonatal mortality rate also decreased due to increased number of institutional delivery.

## 9. Ensuring corner-based health care

Skilled workforce has been developed to provide quality health care. In the Chowgacha UHC, the health care service was provided through different units. In this model health institution, different corners have been established such as for men, women, children, IMCI, ANC PNC, VIA, EPI, breast feeding, ORT, Health Education, NCD etc. In the UHC the indoor is arranged in four units - men, women, children and EOC. Moreover, emergency healthcare services have been ensured by physicians or medical assistants in districts and upazillas on 24/7 basis. It developed the responsibility of the service providers. Due to the availability of corner-based healthcare in the outdoor, patients waiting time have been significantly reduced; overall, there has been

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improvement in illness-based health care services. These has resulted increased number and satisfaction of the patients. Increasing the number of major operations for ensuring the health services of the indoor, increased the number of beds also.

## 10. Birth and death event recording, monitoring progress and ensuring accountability

To monitor the progress of health and family planning activities, it is essential to collect birth and death dara through the UHC. Even though birth data is collected in two of the department, weakness in data collection of death is seen. As a result, it is not possible to know the exact birth and death statistics, birth rate, population growth rate, neonatal death rate, infant mortality rate, maternal mortality ratio, age & disease specific mortality rate. As a result, the responsibility of monitoring the progress of the program and making decisions accordingly becomes difficult, even the accountability could not be ensured. This information was collected primarily since 1995 in Chowgacha UHC. It has shown tremendous success in this year of 2018, 22 years since 1996.

## 11. Health care development by unit management

One of the key features of this model is to ensure the overall service of the patients considering the patient's needs, not considering the health services partially. In this management the service provider does not think that his responsibility is complete only after completing his own part of technical work. The next step is to follow whether the patient is getting the necessary services. Head of a ward considers all aspects of service management until his patients leave from the hospital. The timely attendance of the officers and employees of his unit, cleanliness of the unit, food supply of patients, control visitors in the ward, and all the issues including control of the crowd are considered as a whole. This holistic approach has been given importance in Jhenaidah Hospital.

## 12. Health awareness by health education

Health education programs are vital in health institutions. Various national activities related to change in the behaviour of patients exist, such as health education for maternity services, child health services, adolescent health services, infectious diseases, NCD, family planning, EPI, diarrhoea, pneumonia, rabies etc. and awareness programme to encourage people to utilize the government health services. One of the main goals of health education is so that responsible health officials get information about the problems of the patients and could take necessary action to solve it.

There are two such health education centres in Jhenaidah Sadar Hospital and three at Chowgacha UHC. Each day, the program was conducted by audio visual method. Birganj UHC, has established a library for patients and attendants. The health educators are the highest level

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of staffs at all disciplines of the facility. The health awareness and education program were conducted by responsible officials by rotation through weekly roster.

## 13. Cleanliness (Waste management) and beautification of the facility and premises

The cleanliness of the health facility drives away the danger of infection from the hospital, on the other hand the smell-free environment makes the patients happy. Cleanliness is not for only in the hospital, it is essential to keep the entire premises clean. Organized flower gardens, medicinal plant gardens, nutritional plant gardens, are maintained in the public hospital by hospital management. Cleanliness and beauty of the facility and campus have significantly improved, though sanctioned manpower is not adequate to do the job and maintain. With the help of community financial assistance extra manpower have been recruited and sustained. Due to the introduction of waste disposal system, the hospital has become clean and smell-free. Healthcare providers, including physicians, formally participate in cleanliness drive of the entire hospital and premises on any national day. This drive on a special day is named as "Big Cleaning Day".

## 14. Provision of safe blood transfusion for emergency needs

Safe Blood transfusion is an important component of health care during the operation of any major surgery, especially during the Caesarean operation, ante-partum haemorrhage (APH) and post-partum haemorrhage (PPH). Blood grouping of enthusiastic people started in the Chowgacha UHC to collect blood donor on the Victory Day of 1996. The people were listed by blood grouping, whose residence were within 1.5 kilometre from the UHC. About 600 blood donors were listed. These blood donors are called "Moving blood banks". Since then there have never been a shortage of blood for the patients of Chowgacha UHC. Maternal lives have been saved by emergency blood transfusion. Special efforts have been made on each National Day to collect blood donor. In Durga Puja mondops, initiative was taken to collect blood donor & success was achieved.

Another responsible group was formed to collect blood from the blood donors at night. In 2005, safe blood transfusion unit was established in collaboration with UNICEF for safe blood transfusion at Chowgacha UHC.

## 15. Zero tolerance against corruption

One of the problems of the hospital is irregularities and corruption. People are cooperating doctors and health service providers to tackle such problems. Due to the role of the Community Support Committee, nobody could get any illegal favour from the hospital, whatever influential they might be. So, revenue income has gone up. It is also easier to stop the misappropriation from patients in the name of “thanking”. It has been possible to protect the patients and the facility from the hostile outsiders who posed as “helper”. In issuing medical certificate, illegal

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pressure has been averted, the corruption of the 'brokers' has been successfully prevented by the community support.

In government hospitals, especially at district and upazilla, there is a provision for collecting government determined fees for diagnosis by pathology, x-ray, ECG, ultrasonography and other tests. But opportunity for financial irregularities arises due to absence of cash counter. Government employees and middlemen often become involved in these irregularities. At Jhenaidah, cash counter has started with local initiative. As a result, the annual income of Jhenaidah Hospital has stood at Taka 34 lakhs in 2011. In 2017 it has increased to Taka 72 lakhs. So financial discipline has been improved.

## 16. Implementation of the 5S KAIZEN-TQM for the improvement of health care

The first step of quality service in the hospital are 5S

1st: Sort - Removing unnecessary accessories from workplace

2nd: Set- Keep the necessary accessories properly

3rd: Shine - Maintain cleanliness

4th: Standardized - ensure the quality of the above three works

5th: Sustain - Continuation of the mentioned activities

Kaizen: The Japanese word Kaizen stand for “the means for a change or problem solving process”.

It is possible to make quality service using the 5S KAIZEN-TQM .

## 17. Improvement of financial management by setting cash counter

The patients have to pay government determined fees for diagnostic tests and other services in district and upazilla hospitals, this is one of the important sources of government revenue. Medical Technologists collect sample and fees at the same time, due to manpower shortage, again run laboratory diagnostics test of samples, X-ray etc., distribute reports to patients. It is not unusual to occur financial mismanagement due to this system. Collection of fees from patients and handing of receipts by setting separate cash counter at Jhenaidah Sadar hospital was established to overcome the problem, which stopped the mismanagement.

The number of patients attending the hospital has been increased manifold for providing improved healthcare. And significant improvement in revenue income achieved due to improved financial management ensuring digital technology and discipline. On the other hand, revenue income has been significantly increased due to stoppage of unethical chances.

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## The rationale for the expansion of the Chowgacha-Jhenaidah (C-J) model

After the successful implementation of the Millennium Development Goals (MDG), the progress of the Universal Health Coverage and Sustainable Development Goal (SDG) by 2030, is going forward. It is not possible for the government to implement this huge activity alone, as there are many limitations and challenges. In order to implement this program, the involvement of the community with the government is essential. The C-J model with community engaged management has already been established as an example of health service management. In last 22 years, this model has been successfully going on at Chowgacha UHC in Jashore and for last 6 years in Jhenaidah Sadar Hospital. The program is also being implemented in 15 district and Upazilla hospital of the country.

In order to achieve SDG and universal health coverage in Bangladesh within scheduled time, the C-J community engaged health management model will be replicated and implemented in the upazilla and district level throughout the country, based on evidence, time-bound and initiatives. It is to be noted that, in 2010, Director General of World Health Organization (WHO) visited to Chowgacha UHC and recommended to replicate the model throughout the country. An official order from the Hon'ble Prime Minister's Office has been issued to replicate C-J Model nationwide.

## Vision

To build up a healthy population in Bangladesh

## Mission

To ensure universal health coverage for every citizen in Bangladesh within 2030

## Goal

Replication of C-J Model within 2022 at all Upazilla Health Complexes and District Sadar Hospitals in Bangladesh for quality improvement of healthcare

## Target

By June 2020: 27 Upazilla and 18 district hospital

By June 2021: next 250 Upazillas and 35 districts hospitals

By the end of June 2022: all the remaining Upazillas and district hospitals

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## Objectives

1. To take steps at the local level to achieve the goal of universal health coverage for health development and sustainable development targets of the 7th Five Year Plan of Bangladesh Government
2. To ensure optimum utilization of benefits utilizing resources and opportunities allocated by the government
3. To ensure local community participation in local health management and opportunities for multifaceted contributions
4. To develop leadership quality and motivation
5. To improve health care through unit-wise service
6. To develop of various committees, for drafting the public health care plans, implementation, management and monitoring at local level
7. To maintain consistent success, development and stability through regular monitoring and evaluation

## Strategies

1. Nationwide baseline survey of all UHC and District Hospitals for phase wise expansion of C-J model
2. Selection of upazilla and district hospitals for phase-wise implementation of the community engaged public health system
3. Preparation of planning at national level for the expansion of this model and the formation of committees at different level
4. Promote nationwide publicity and social movement in expanding the public health model of Chowgacha - Jhenaidah (C-J)
5. Leadership and management development training for selected Upazilla health officers and practitioners of the district hospitals
6. Ensuring Upazilla Health officials and District Hospital's required manpower
7. Arranging supply of necessary provisions in selected UHCs and district hospitals
8. Resources and logistics allocated at government health facilities - ensuring optimum utilization and replenishment
9. Creating opportunities for community participation and multifaceted contributions in health management at local level
10. The committee will work at the local level, by forming groups, to implement community engaged public health system
11. Sustain institutional successive success, development and stability through the implementation and management of the government's plans at local level and regular monitoring and evaluation

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## Chowgacha Jhenaidah (C-J) Community Engaged Health Management Model expansion committees

1. C-J Community Engaged Health Management National Steering Committee
2. C-J Community Engaged Health Management Model National Technical Committee
3. C-J Community Engaged Health Management Model Community Support Committee in District Hospital
4. C-J Community Engaged Health Management model in District Hospital Implementation committee
5. C-J Community Engaged Health Management Community Support Committee in UHC
6. C-J Community Engaged Health Management implementation committee in UHC

### 1. CJ Model Community Engaged Health Service National Steering Committee

| Serial | Designation | Position in Committee |
|---|---|---|
| 1. | Secretary, Health service division, MOHFPW | Chairperson |
| 2. | Additional Directorate General (Administration), DGHS | Member Secretary |
| 3. | Directorate General, DGHS | Member |
| 4. | Directorate General, Family planning, DGFP | Member |
| 5. | Directorate General (Nursing) | Member |
| 6. | Additional Secretary (Public Health), Health service division, MOHFPW | Member |
| 7. | Additional Secretary (Hospital) Health service division, MOHFPW | Member |
| 8. | Additional Secretary, (Budget, Evaluation and Activities), Ministry of Social Welfare | Member |
| 9. | Directorate General (HEU), MOH & FW | Member |
| 10. | Directorate General (Admin), Ministry of Local Govt. | Member |
| 11. | Line Director (HMS) & Director (Hospital) DGHS. | |
| 12. | Member Secretary CJ community engaged health management national technical committee | Member |
| 13. | -------- | |
| 14. | ---------- | |
| 15. | ---------- | |

## Responsibilities

1. Approve policy and strategic plan
2. Approve recommendations of CJ Model national technical committee
3. Approve and allocation of budget

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4. At least two meeting should be held in every year
5. New member may be co-opted, if needed

## 2. C-J Model Community Engaged Health management national technical committee

| Serial | Designation | Position in Committee |
|---|---|---|
| 1. | Director General of Health Services, DGHS | Advisor |
| 2. | Additional Directorate General (Administration) | Chairperson |
| 3. | Chief Coordinator, Coordination and Support Centre, DGHS | Member Secretary |
| 4. | Director (Administration), DGHS | Member |
| 5. | Director (Admin), DG Nursing | Members |
| 6. | Director (Finance), DGHS | Member |
| 7. | Director (Hospital), DGHS | Member |
| 8. | Director (Primary Health Care), DGHS | Member |
| 9. | Line Director, Community Based Health Care, DGHS | Member |
| 10. | Assistant Director (Coordination), DGHS | Member |
| 11. | Focal Person, QIS Secretariar, Health Economics Unit, MoHFW | Member |
| 12. | Senior Scientist, Health System and Population Science Division, icddrb | Member |
| 13. | Representative, UNICEF, Dhaka | Member |
| 14. | Representative, WHO, Dhaka, Bangladesh | Member |
| 15. | Representative, JICA, Dhaka | Member |
| 16. | Representative of Bangladesh Medical Association (BMA) | Member |
| 17. | Dr. M. Mushtuq Husain, Coordination and Support Centre, DGHS | Member |
| 18. | Dr. Md. Emdadul Haque, Senior Consultant Gynae, Jhenaidah Sadar Hospital [PRL] | Member and Consultant |
| 19. | Medical Officer (3), Coordination and Support Centre, Health Dept. | Member |

### Responsibilities:

1. Replication of Chowgacha Jhenaidah community engaged health management model
2. Preparation of action plan, order, SOP, Apps, monitoring form related to the model
3. Evaluation, analysis and preparation of summary note ordered by MoHFW
4. Preparation of training manual
5. Organize training
6. Play role in improving the quality of upazilla and district hospital

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7. Follow the order of DGHS related to the model
8. The meeting will be held at least once time per month, or more if necessary
9. The committee may co-opt members, if needed

## 3. C-J Model Community engaged district Sadar Hospital Community Support Committee

| SL NO | Designation | Rank in committee |
|---|---|---|
| 1. | Mayor, Municipality of district town | Chairperson |
| 2. | Civil Surgeon | Co ordinator |
| 3. | Super. District Hospital | Member Secretary |
| 4. | Representative of honourable MP | Member |
| 5. | Representative of Zilla Parisad Chairman | Member |
| 6 | Representative of Deputy Comissioner | Member |
| 7. | Representative of Superintendent of Police | Member |
| 8. | Representative of Deputy Director Social Welfare | Member |
| 9. | Representative of BMA | Member |
| 10. | Representative of Bar Association | Member |
| 11. | Representative of Press Club (nominated by DC) | Member |
| 12. | Representative of NGO (nominated by DC) | Member |
| 13. | Representative of Civil Society (nominated by DC) | Member |
| 14. | RMO | Member/ Focal person |
| 15. | Consultant (1) | Member |

### Responsibilities:

1. Collection of information about C-J model to prepare an action plan to implement it
2. To assist the hospital authorities in improving the quality of care, healthy environment, cleaning & security
3. To assist the hospital authorities for optimum utilization of govt. resources and its maintenance
4. To fulfil the demand & development of the hospital in accessing govt. resources, create a community support fund so that donors, organization can donate to the fund
5. Identify the problems of health services and play a role to solve the problems locally

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6. Encourage the doctors, nurses and other service providers in hospital services and giving constructive suggestion
7. The committee should sit in a meeting in every month and co-opt members
8. Form sub committee if needed

## 4. C-J Model Implementation Committee in district Sadar Hospital

| SL NO | Designation | Rank in committee |
|---|---|---|
| **1.** | Super, District Sadar Hospital | Chairperson |
| **2.** | Residential Medical Officer | Member Secretary |
| **3.** | Different WIT facilitator and team leaders | Member |

### Responsibilities:

1. Review the progress of the different WIT
2. Make an action plan for the next month
3. Report to the higher authority in every month in a prescribed format
4. Request to the community support committee in the form of proposals by determining which assistance is required from the community support committee
5. Approval of the way of solution of different problems in WIT
6. The meeting of the committee should be held once in a month

## 5. C-J Model Upazilla Health Complex Community Support Committee

| Sl no | Designation | Rank in Committee |
|---|---|---|
| 1. | Mayor, Municipality* | Chair person |
| 2. | Upazilla Health and Family Planning Officer | Member Secretary |
| 3. | Representative of the Honourable M P (1) | Member |
| 4 | Representative of Upazilla Chairman (1) | Member |
| 5. | Representative of UNO (1) | Member |
| 6. | Representatives of the union council's chairmen (1) | Member |
| 7. | Representative of OC (1) | Member |
| 8. | Upazilla Social Welfare Officer (1) | Member |
| 9. | Representative of Press club (1) | Member |
| 10. | Representative of NGO (UNO nominated) (1) | Member |
| 11. | Representative of Civil Society (UNO nominated) (1) | Member |
| 12. | Consultant (1) | Member |

| Sl no | Designation | Rank in Committee |
|---|---|---|
| 13. | Residential Medical Officer (1) | Member |

*Where Paurasava do not exist, Upazilla Chairperson will be the Chairperson of the Committee

### Responsibilities:

1. Collection of information about C-J model to prepare an action plan to implement it
2. To assist the hospital authorities in improving the quality of care, healthy environment, cleaning & security
3. To assist the hospital authorities in optimum utilization govt. resources and its maintenance
4. To fulfil the demand & development of the hospital in accessing govt. resources, create a community support fund so that donors, organization can donate in the fund
5. Identify the problems of health services and play a role to solve the problems locally
6. Encourage the doctors, nurses and other service providers in hospital services and giving constructive suggestion
7. May co-opt any member and form any sub-committee, if needed
8. The committee should perform a meeting in every month

## 6. C-J Model Implementation Committee in Upazilla Health Complex

| Sl No | Designation | Rank in Committee |
|---|---|---|
| 1 | UH&FPO | Chairperson |
| 2 | RMO | Member Secretary and Focal Person |
| 3 | Team Leaders and Facilitators of WIT Committees | Member |

### Responsibilities:

1. Review the progress of the different WIT
2. Make an action plan for the next month.
3. Report to the higher authority in every month in a prescribed format
4. Request to the community support committee in the form of proposals by determining which assistance is required from the community support committee
5. Approval of the way of solution of different problems in WIT
6. The meeting of the committee should be held once in a month

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## Action plan of National Strategy for Implementation of C-J model

| No | Work | Implementer | Duration | Comment |
|---|---|---|---|---|
| 1 | Formation of national Technical Committee | DGHS | - | Completed |
| 2 | Formation of the National Steering Committee | MOH&FW | December 18 | - |
| 3 | Formation of expert groups for implementation | National Technical Committee | December 18 | - |
| 4 | Primary survey nationwide | National Technical Committee, Coordination and support Centre, DGHS | - | Completed |
| 5 | Visit of community supported health institutions | National Technical Committee | | Completed |
| 6 | Preparation of strategies | National Technical Committee, Coordination and Support centre (DGHS) | - | Running |
| 7 | Finalization of strategy | National Steering Committee | December' 18 | - |
| 8 | First phase of district-Upazilla UHC identification | National Technical Committee | - | Completed |
| 9 | collecting and analysing primary data of district and Upazilla hospital in the first phase, | National Technical Committee | December' 18-January' 19 | Running |
| 10 | Prepare the Training Manual | National Technical Committee | December' 18-January' 19 | - |
| 11 | Motivational program starts in Field level by all stakeholders. | National Technical Committee, Director (CBHC) | January' 19 | - |
| 12 | Starting for Implementation of C-J Model in first phase, district and Upazilla Hospital | Coordination and Support Centre DGHS, Director CBHC | January' 19 | - |

| No | Work | Implementer | Duration | Comment |
|---|---|---|---|---|
| 13 | Formation of Community Support Groups | Civil Surgeon and superintendent of District Hospital of the concerned district | January' 19 | - |
| 14 | Starting of second phase activities | Department of Health Coordination and Assistance centre, CBHC | March' 19 | - |
| 15 | Sustainability of Chowgacha UHC and Jhenaidah Sadar Hospital | National Technical Committee and Community Support Committee | March' 19 | Running |

## Institutional implementation Action plan

| Sl No | Program | Responsibilities | Duration |
|---|---|---|---|
| 1 | Formation of Community Support Committee | Local Hospital/ Upazilla Health Complex Authority | 15 Day |
| 2 | Preparation of initial assessment report | QIC and WIT team | 30 Day |
| 3 | Identification of Problems | QIC and WIT team | 30 Day |
| 4 | Determining the vision, goals and goals | QIC team | 15 Day |
| 5 | Motivation through the workshop to achieve the commitment of the people and the service providers to implement the goals | Local Hospital/ Upazilla Health Complex Authority | 90 Day |
| 6 | Preparation and collection of necessary materials | Local Hospital/ Upazilla Health Complex Authority | 180 Day |
| 7 | Prepare policies for building value-based work-environment a institutional culture | QIC Team | 15 Day |
| 8 | Communication with all stakeholders to develop social movements to achieve the goal | Community Support Group and Local Health Authority | 60 Day |
| 9 | Training on quality development 5S KAIZEN-TQM. | Local Health Authority | 7 Day |
| 10 - | Achieve practical knowledge through a team visiting the Chowgacha UHC and Jhenaidah Sadar Hospital | National Technical Committee | 3 days |

| Sl No | Program | Responsibilities | Duration |
|---|---|---|---|
| 11 | An expert team of the model would visit the hospital and develop them by hands-on | National Technical Committee | 7 Day |
| 12 | Evaluate the progress of the program | Local health authority | Every 90 days intervals |

## Conclusion

Healthcare is a multidimensional humanized system. The preventive, curative, promotive, rehabilitative & palliative healthcare of the people is a huge task. It is not possible to implement it by government only. So, community engagement is essential. Chowgacha-Jhenaidah model is a brilliant example of community engaged public health management. It has been possible to achieve public confidence by launching this management. In this system people got the opportunity to be involved in health care, planning and implementation. As a result of community engagement, it will be easier in the future to identify health issues and to help solve problems in the light of their real experiences. This model in Bangladesh will play a significant role in the implementation of the Universal Health Coverage program announced by the World Health Organization.

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## Annexure

**Table 1. Milestone of the development of the Chowgacha model**

| | |
|---|---|
| 1985 | UHC extends to nearby public library and opens six bedded indoor with the help of Upazilla parisad |
| 1989 | National award for success in the EPI program |
| 1994 | The UHC shifted to 31-bedded new Upazilla Health Complex building |
| 1995 | Starting the ORT Corner and starting of Frontline MCH program, and with the help of JICA |
| 1996 | ANC corner, and under five corner starts |
| 1998 | In January a baseline Survey was conducted and made a report 1997 |
| 2000 | Comprehensive EOC program starts with the help of UNICEF |
| 2001 | A plan 2010 for achieving MCH target by 2010 |
| 2004-2014 | Best Performance Award for EOC Services |
| 2006 | Safe Blood Transfusion Unit was opened with support of UNICEF |
| 2007 | Introducing of DSF program for poor pregnant mother |
| 2008 | Recognized as a Women Friendly Hospital |
| 2008 June | Upgraded to 50 bedded and declared as a Model UHC by MOH&FW |
| 2009 | Achieving MDG 5 and 6 |
| 2010 | Director General of WHO visited Chowgacha UHC and recommends for replication |
| 2011 1st January | The success of achieving targets of 2010 and the launch of prevention of non-communicable disease (NCD) programs |
| 2011 July | 5S KAIZEN-TQM program starts by support of JICA |
| 2017 | Awarded Health Minister's Award as stood 1st and also won prize for EOC |
| 2018 | MOH&FW completed the plan for 100 bedded hospital |

## Table 2. Milestone of the activities of the Jhenaidah Model

| | |
|---|---|
| 2012 | Starting of Model activities |
| 2013 | Kaizen Role Model Award for Model |
| 2014 | Jhenaidah team visits Satkhira Sadar Hospital, Awarded second prize for TQM |
| 2015 | Ministry initiates Piloting of District-level Modelling and achieving the best prize for TQM |
| 2016 | Replication of Jhenaidah Model in 10 district hospital&5 Upazilla UHC. Awarded, Health Ministers Award and Best performance Award for EOC |
| 2017 | Awarded Health Minister's award and best performance award for EOC |

## Table 3. Chowgacha Health Complex – improvement evaluation (1995-2017)

| Indicator | Number/ rate | 1995 | 1996 | 1997 | 2000 | 2003 | 2007 | 2011 | 2017 |
|---|---|---|---|---|---|---|---|---|---|
| Bed Occupancy Rate | % | - | - | 78.9 | 93.5 | 88.9 | 201 | 208 | 171 |
| ANC (First Visit) | % (N) | - | 16.3 (817) | 24.6 (1228) | 59 (2861) | 88 (4484) | 132 (7292) | 170 (8935) | 130 (6632) |
| Hospital Delivery Rate | % (N) | - | 2.1 (99) | 3.4 (151) | 6.4 (422) | 15 (661) | 38.1 (1899) | 98.4 (4413) | 66 (2932) |
| C-Section rate | % (N) | - | 0.02 (1) | 0.0 | 0.07 (3) | 0.8 (36) | 9.6 (473) | 22.5 (1107) | 17 |
| Population Increase rate | % | 1.87 | 1.86 | 1.66 | 1.7 | 1.57 | - | 1.36 | 1.17 |
| Maternal Mortality Rate/100,000 | Per live birth | 599 | 300 | 206 | 168 | 114 | 160 | 68 | 91 |
| Neonatal mortality rate/ 1000 | | 42 | 47 | 42.4 | 23 | 28 | 21.5 | 18.2 | 16.6 |
| Infant mortality rate/ 1000 | | 65 | 58 | 56 | 36 | 34 | 29.2 | 23.6 | 18.6 |
| Under 5 mortality rate/ 1000 | | 78 | 67 | 62 | 44 | 43 | 33.3 | 25.7 | 21.1 |
| Low birth weight rate | % | - | - | 36.5 (74) | 15.2 (288) | 13.7 (636) | - | - | - |

Ref. UH&FPO, Chowgacha, Jessore # # #

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## Table 4. Jhenaidah Sadar Hospital- Improvement Evaluation (1998-2017)

| Serial | Subject | 1998 | 2011 | 2012 | 2015 | 2017 |
|---|---|---|---|---|---|---|
| 1 | Emergency & Out Door patient number | 81795 | 170854 | 171070 | 277301 | 340932 |
| 2 | Indoor patients | 12973 | 22435 | 22951 | 29562 | 35569 |
| 3 | Bed occupancy rate% | 126 | 146 | 143 | 193 | 242 |
| 4 | Number of EmOC service | 256 | 1354 | 2268 | 13251 | 15360 |
| 5 | Number of deliveries | 1106 | 2375 | 2379 | 3034 | 2997 |
| 6 | Operation (Major) | - | 780 | 977 | 1582 | 1578 |
| 7 | Revenue income | - | - | - | 60,44,715 | 70,59,560 |

Ref. Superintendent, Sadar Hospital, Jhenaidah

## Table 5. Comparative Data of best performing EOC center of Khulna Division in 2017

| Serial NO | Facility | Type of EOC Service | | | | | |
|---|---|---|---|---|---|---|---|
| | | Normal | | Caesarean | | Total | |
| | | 2011 | 2017 | 2011 | 2017 | 2011 | 2017 |
| 1 | Chowgacha Health Complex | 3406 | 2163 | 1007 | 759 | 4413 (First) | 2922 (Third) |
| 2 | Khulna Medical College Hospital | 1657 | 1448 | 1356 | 1365 | 3013 (Second) | 2524 |
| 3 | 250 Bedded Jessore General Hospital | 1354 | 1172 | 1498 | 1778 | 2852 (Third) | 2950 (Second) |
| 4 | 250 Bedded Kashia General Hospital | 1762 | 1595 | 915 | 919 | 2677 | 2514 |
| 5 | 100 Bedded Jhenaidah Sadar Hospital | 1947 | 1854 | 430 | 1143 | 2337 | 2997 (First) |

Ref. MIS, DGHS

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## Table 6. Comparative data of different countries resources provided by the respective governments

| Sl No | Country | Per capita health Expenditure per year by Govt. ($) | Doctors per 10000 Population | Nurse per 10000 Population | Beds per 10000 population | Out of pocket expenditure % |
|---|---|---|---|---|---|---|
| 01 | USA | 9536 | 25.5 | 99 | 37 | 11.08 |
| 02 | UK | 4356 | 28.0 | 84 | 30 | 14.79 |
| 03 | Malaysia | 386 | 12.8 | 34 | 18 | 36.67 |
| 04 | Sri Lanka | 118 | 7.3 | 17.5 | 31 | 38.43 |
| 05 | India | 63 | 7.3 | 20.4 | 09 | 65.06 |
| 06 | Nepal | 44 | 08 | 4.7 | - | 60.41 |
| 07 | Pakistan | 38 | 08 | 06 | 06 | 66.40 |
| 08 | Bangladesh | 37 | -- | -- | -- | 67.00 |

Ref: www.worldbank.org Accessed on 20/10/2018

## Table.7. 7th Five Year Plan, Health, Nutrition and Population Target

| SL No. | Indicator (effect/ results) | Start Year Info (Start Year) | The target of 7th Five-Year Plan is only by 2022 | SDG by 2030 |
|---|---|---|---|---|
| 1 | life expectancy at birth (in year) | 70.4 (SVRS 2013) | 72 | |
| 2 | Under five mortality rate (per 1000 live birth) | 46 (BDHS 2014) | 37 | 25 |
| 3 | Infant (0-1 yr) mortality rate (per 1000 live births) | 38 (BDHS 2014) | 20 | |
| 4 | Maternal mortality ratio (per 100000 live birth) | 170 (MMEIG 2013) | 105 | 70 |
| 5 | The ratio of underweight children under 5 years (%) | 32.6 (BDHS 2014) | 20 | |
| 6 | Ratio of stunting in under 5 years children (%) | 36.1 (BDHS 2014) | 25 | |
| 7 | Total fertility rate (children per head of every woman of reproductive age) | 2.3 (BDHS 2014) | 2.0 | |
| 8 | Contraceptive prevalence rate (%) | 62.4 (BDHS | 75 | |

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| SL No. | Indicator (effect/ results) | Start Year Info (Start Year) | The target of 7th Five-Year Plan is only by 2022 | SDG by 2030 |
|---|---|---|---|---|
| | | 2014) | | |
| 9 | Immunization coverage among Children under the age of one year | 78 (BDHS) | 95 | |
| 10 | Ratio of childbirth (%) delivery by trained person | 42.1 (BDHS 2014) | 65 | |
| 11 | Rate of institutional delivery according to wealth status (ratio of highest and lowest fifth) | 15:69.5 (BDHS 2014) | 1:3.5 | |
| 12 | Tuberculosis identification rate (%) | 53 (GTR 2014) | 75 | |
| 13 | Neonatal mortality rate per 1000 live birth | 25 (2014) | 18 | 12 |
| 14 | The wasting rate of children aged under five years | 14 | 12 | |
| 15 | Ante-natal Health Care (ANC) 4th visit | 31.2 | 50 | |
| 16 | Exclusive Breastfeeding infant under 6 months | 55.3 (2014) | | |
| 17 | Adolescent fertility rate (15-19) per 1000 married couple | 31 | 25 | |
| 18 | Deaths due to NCD per ten thousand population (30 - 69 years) | | | |
| 18. a | Heart disease | | | |
| 18 b | Cancer | | | |
| 18 c | Diabetes | | | |
| 18 d | Chronic respiratory illness | | | |
| 19 | Mortality in road traffic accidents (per 100,000 population) | | | |
| 19a. | Male | | | |
| 19 b. | Women | | | |
| 20. | Smoker's rate (every 100,000 population) in 15+ years | | | |

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## Table. 8 Health related SDG goals, target (13), and Indicator (27)

| SL No. | Target | Indicator | Index (effect / results) | Information Year Information Year (Start Year) | Target by 2030 |
|---|---|---|---|---|---|
| 1 | 3.1 | 3.1.1 | Maternal mortality rate (per 100,000 live births) | 170 (MMEI 2013) | 70 |
| 2 | | 3.1.2 | Percentage of Delivery by physicians and trained person | 42.1 (BDHS 2014) | |
| 3 | 3.2 | 3.2.1 | Under 5 Mortality rate / 1000 live births | 46 (2014) | 25 |
| 4 | | 3.2.2 | Neonatal death rate/1000 live births | 25(2014) | 12 |
| 5 | 3.3 | 3.3.1 | HIV/AIDS incidence rate | 0.01% | |
| 6 | | 3.3.2 | TB Case detection rate | 53 %(2014) | 100% |
| 7 | | 3.3.3 | Malaria incidence rate (High endemic area) 3 district | 1-10/10,000pop | No public health problem |
| 8 | | 3.3.4 | Hepatitis B incidence rate | | |
| 9 | 3.4 | 3.4.1 | Death due to NCD (30-69)/10,000 | 18 (WHO-2014) | 6 |
| | | 1 | Heart Disease | | |
| | | 2 | Cancer | | |
| | | 3 | Diabetes | | |
| | | 4 | Ch Resp disease | | |
| 10 | | 3.4.2 | Incidence of suicide/100,000 pop | 8(2014) | 2.67 |
| | | 1 | Male | | |
| | | 2 | Female | | |
| 11 | 3.5 | 3.5.1 | Incidence of drug addiction/10,000pop | | |
| 12 | 3.6 | 3.6.1 | Mortality rate due to RTA/100,000 pop | 85.6(World Bank-2002) | ½ in 2020 |
| | | | Male | | |
| | | | Female | | |
| 13 | 3.7 | 3.7.1 | CPR | 62.4(2014) | |
| 14 | | 3.7.2 | Fertility rate (15-19)/1000 couple | 31 | |
| 15 | 3.8 | | Universal Health Coverage/ 1000 pop | | |
| 16 | 3.9 | 3.9.1 | Death rate due to water, air, soil pollution/10,000 | | |

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| SL No. | Target | Indicator | Index (effect / results) | Information Year Information Year (Start Year) | Target by 2030 |
|---|---|---|---|---|---|
| | | | pop | | |
| 17 | 3a | | Tabaco use rate/1000 pop (15+yr) | 25(WHO2014) | |
| | | | Male | 48 | |
| | | | Female | 02 | |
| 18 | 3b | 3b1 | Essential drugs for all | 66%(2014) | 100% |
| | | 3b2 | Vaccine for all | | |
| | | 3c3 | Family planning commodities supply | 84.4(2014) | |
| 19 | 3.c | 3.c | Health Manpower/100,000 pop | | |
| | | 1 | Medical Officer | | |
| | | 2 | Consultant (Clinical) | | |
| | | 3 | Consultant (Preventive) | | |
| | | 4 | Nurse | | |
| | | 5 | Midwife | | |
| | | 6 | Public health nurse | | |
| | | 7 | Sanitary inspector | | |
| 20 | 3.d | 3.d | Capabilities of disaster management | | |
| | | 3.d.1 | Forecasting Capacities of disaster | | |
| | | 3.d.2 | Capacities of risk reduction | | |
| | | 3.d.3 | Management development capacity building | | |

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